

听神经瘤手术中的核心技术

长江航运总医院 武汉脑科医院

岑波

手术体位

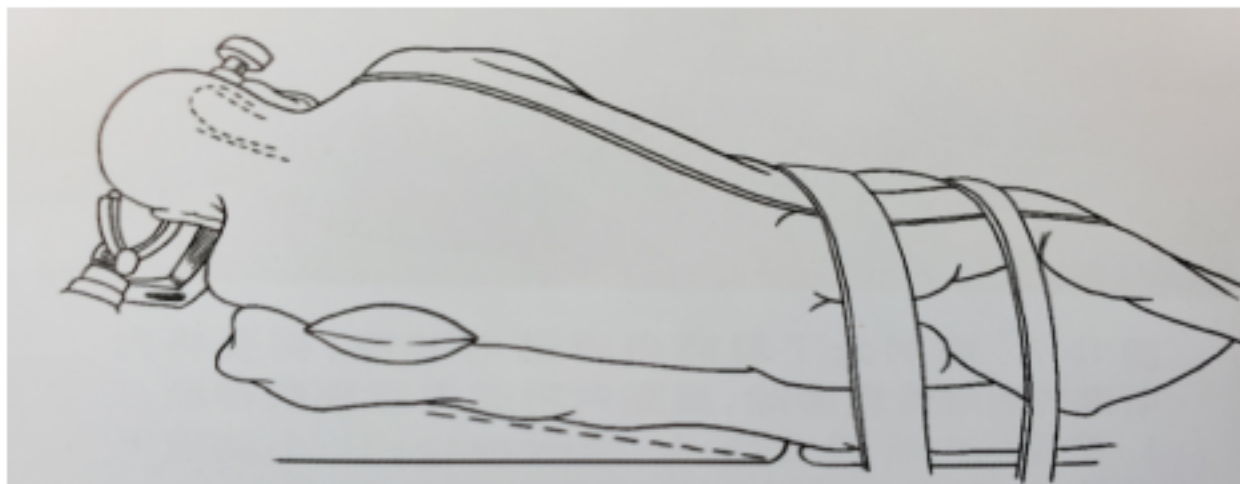
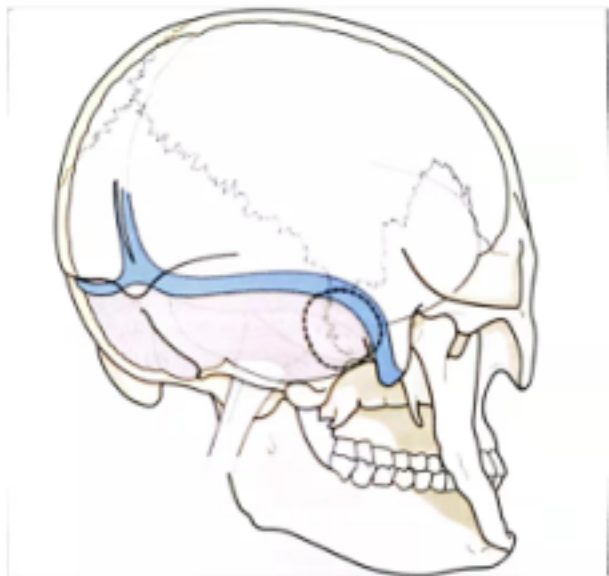


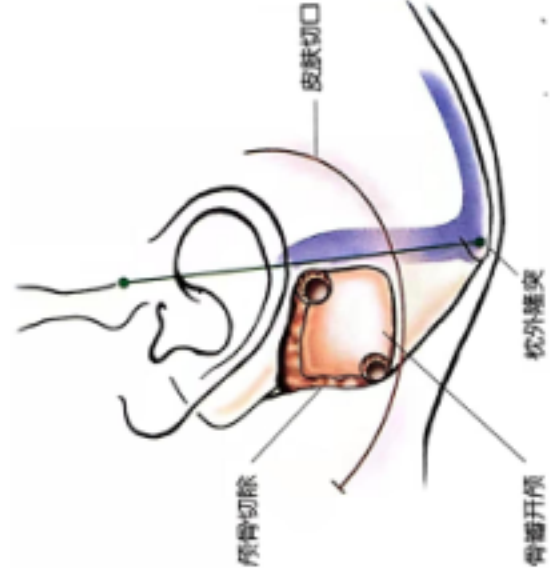
图 15-1 手术体位常为侧俯卧位,病变侧朝上,该体位对桥小脑角区暴露好,并能够有效防止空气栓塞

开颅要点

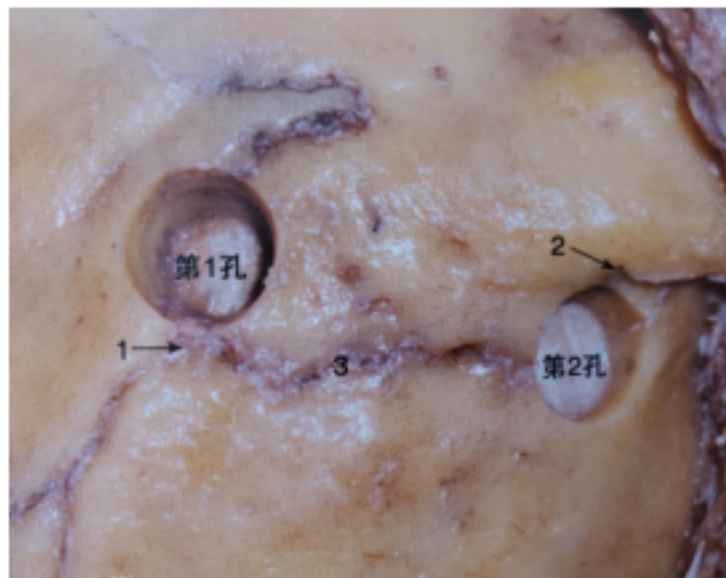
- 关键孔的寻找
- 横窦及乙状窦的显露与保护
- 乳突导静脉的处理
- 椎动脉的保护

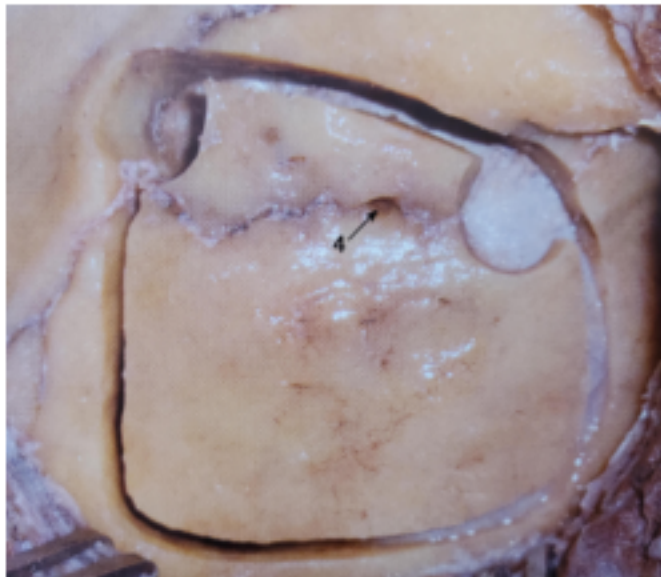
关键孔的位置

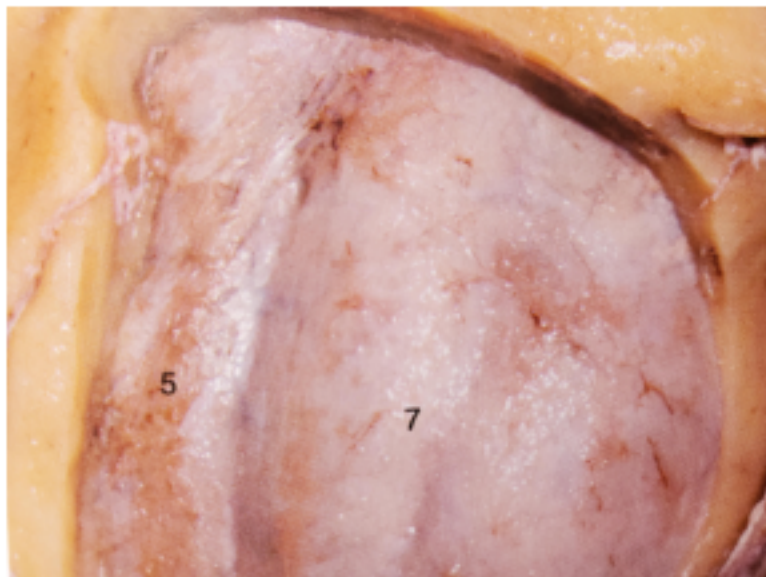


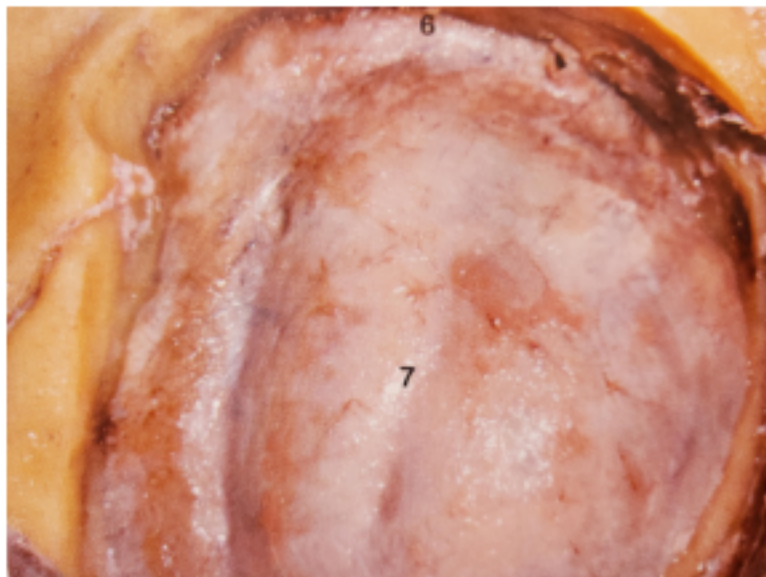


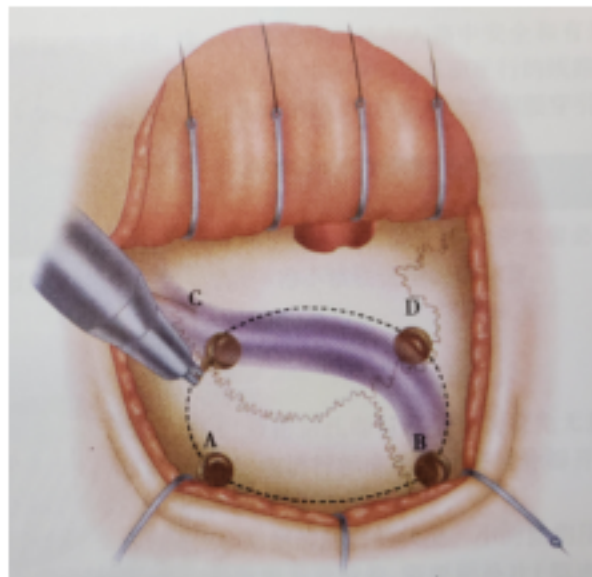




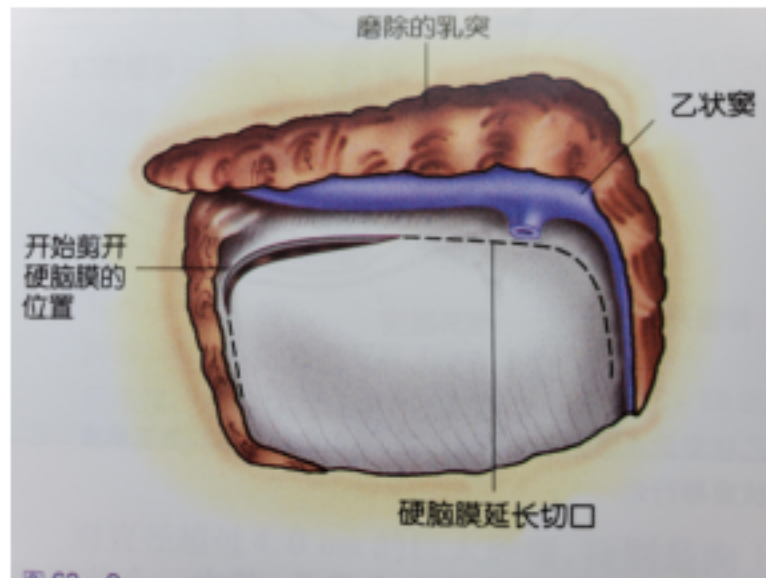




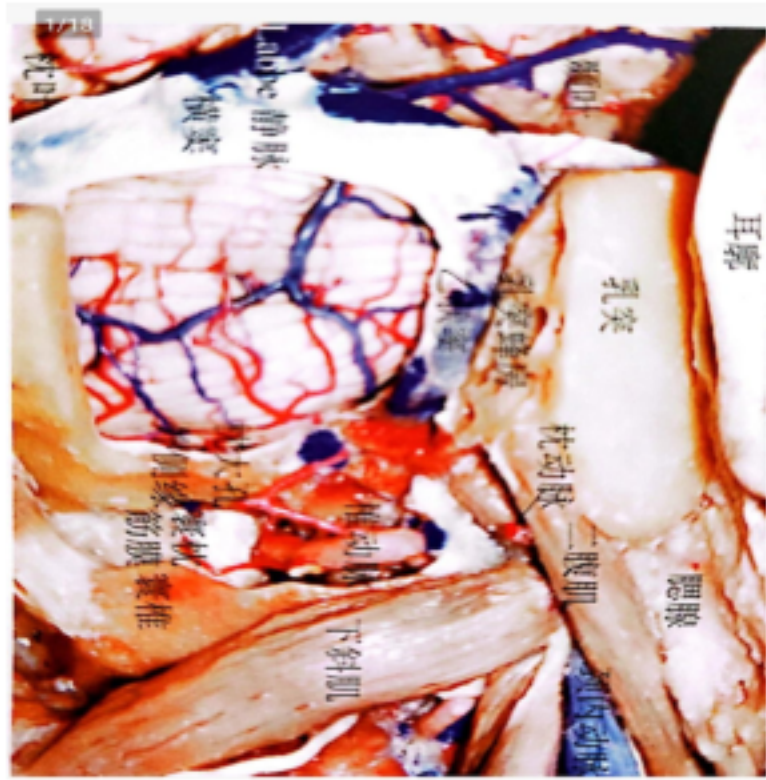




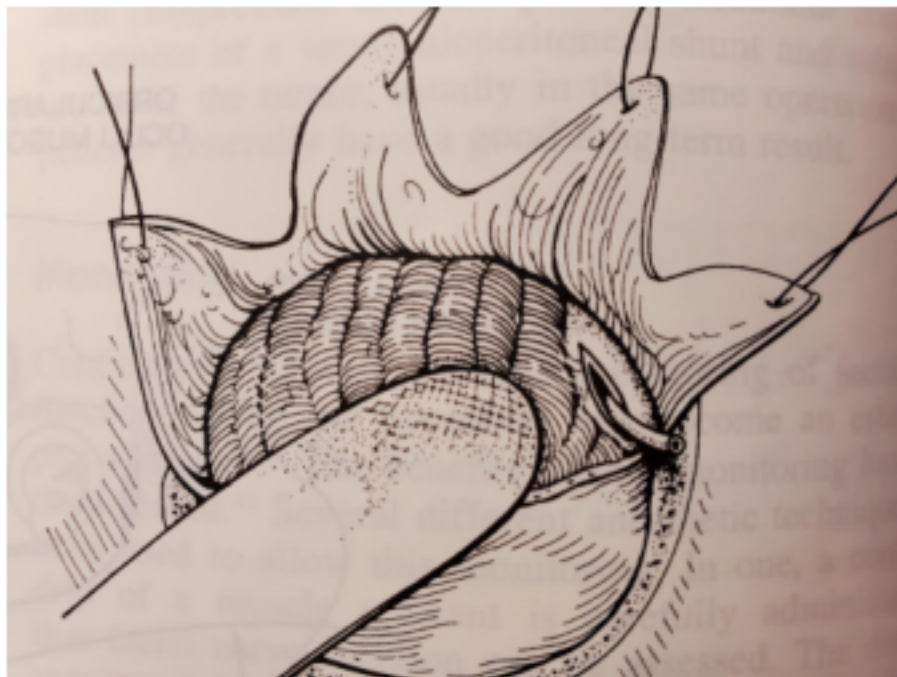
横窦及乙状窦的显露与保护



椎动脉的保护



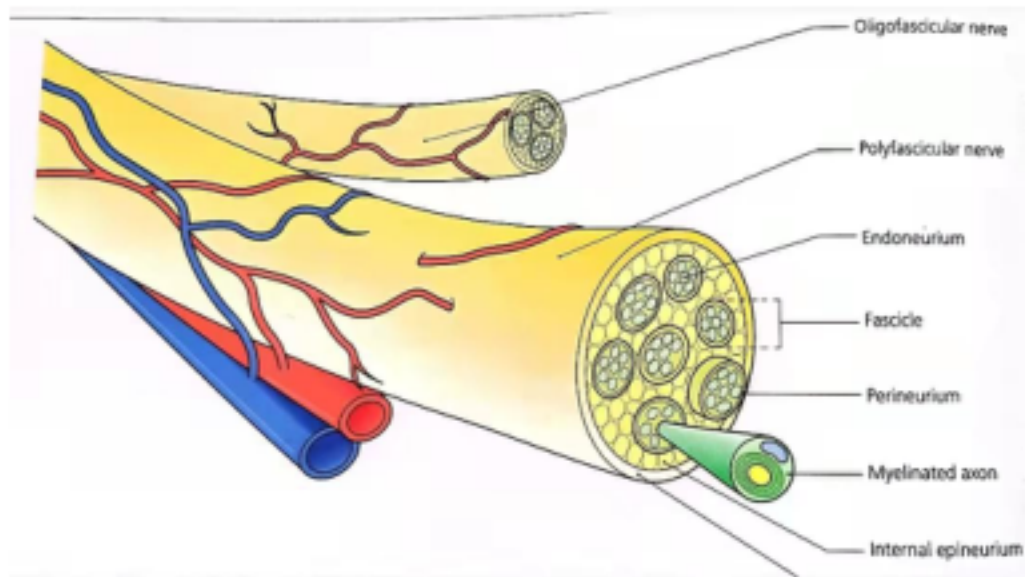
硬膜剪开的方式



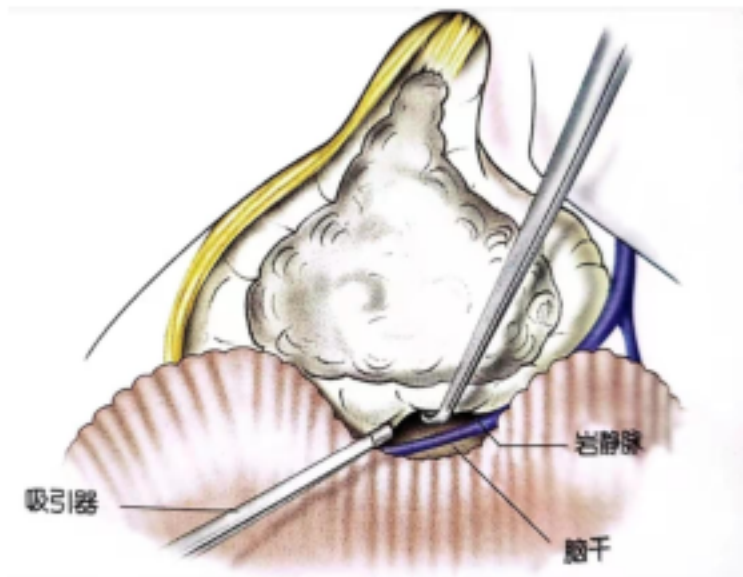
高颅压的控制

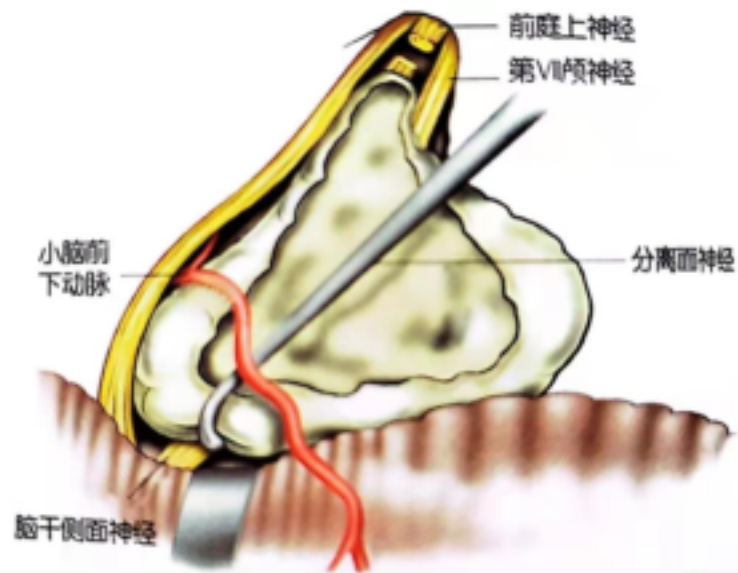
- 甘露醇快速静滴
- 过度换气
- 枕大池及小脑延髓池
- 脑室外引流

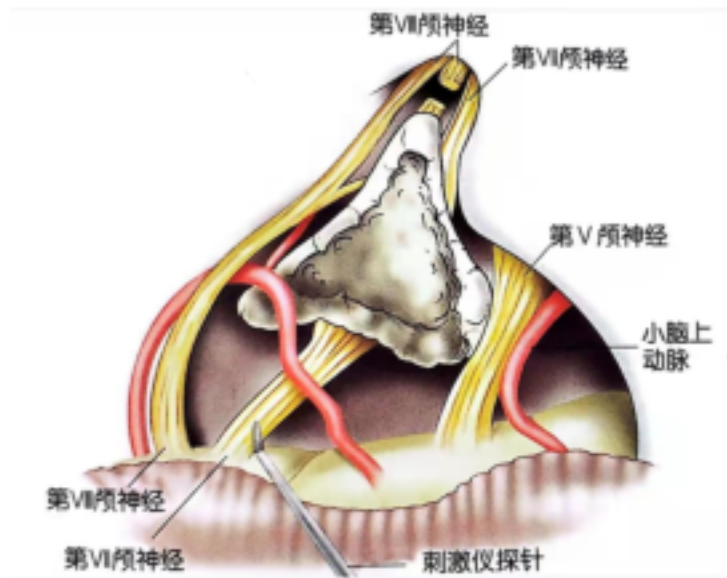
肿瘤固有包膜的寻找



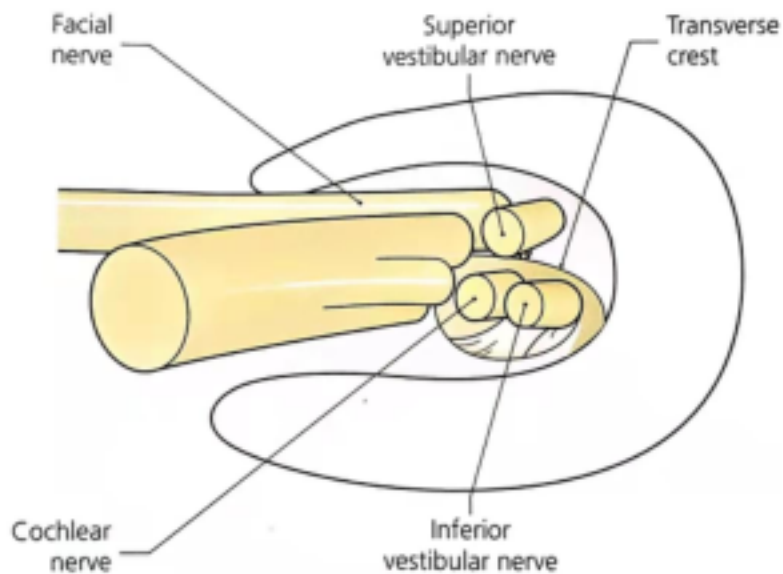
肿瘤表面神经血管的分离与保护



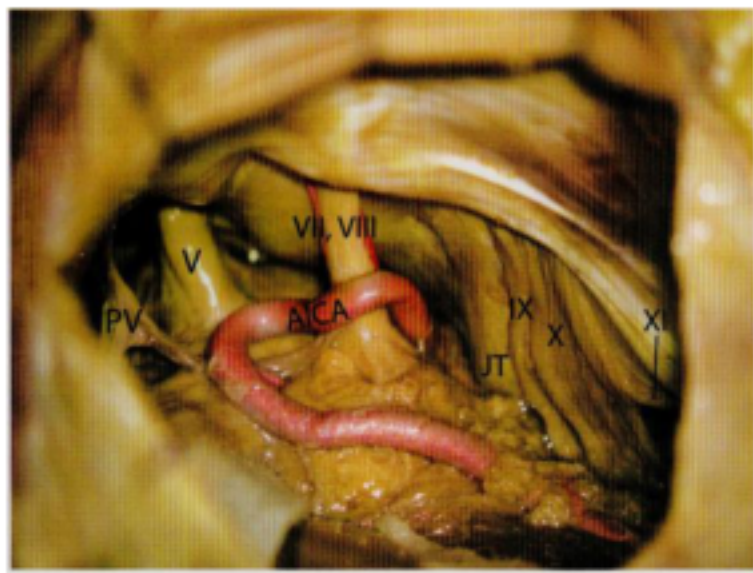




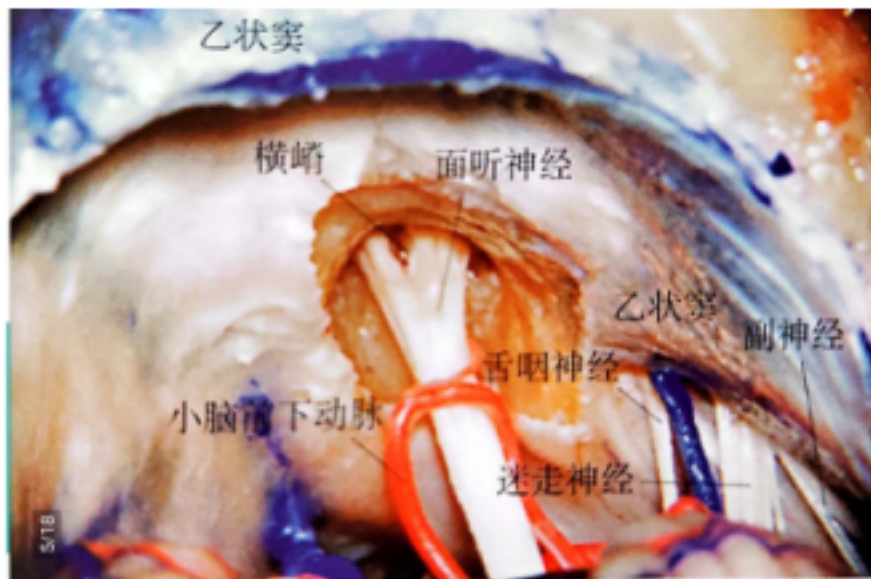
面神经的寻找与保护



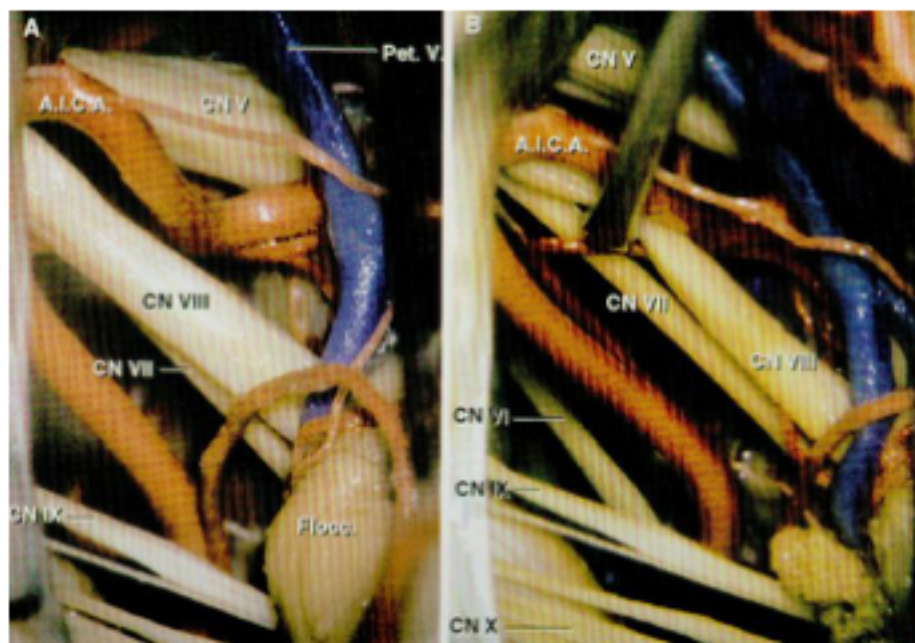
内听道的寻找与开放

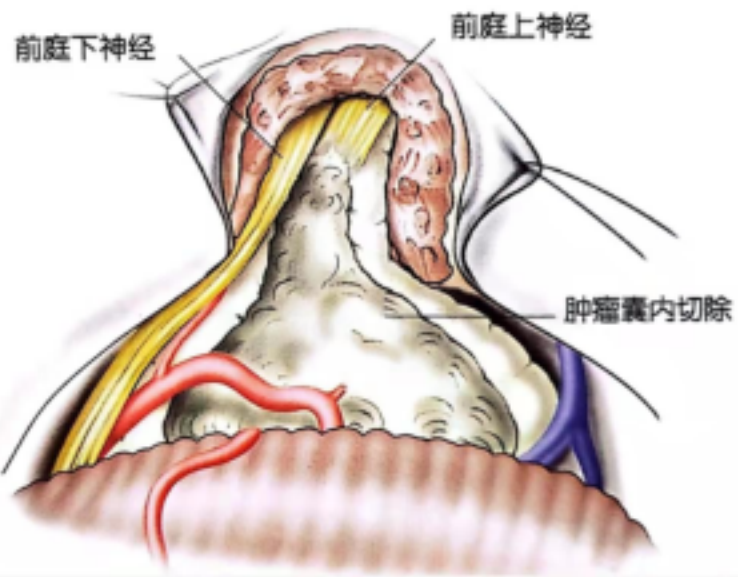


内听道的寻找与开放

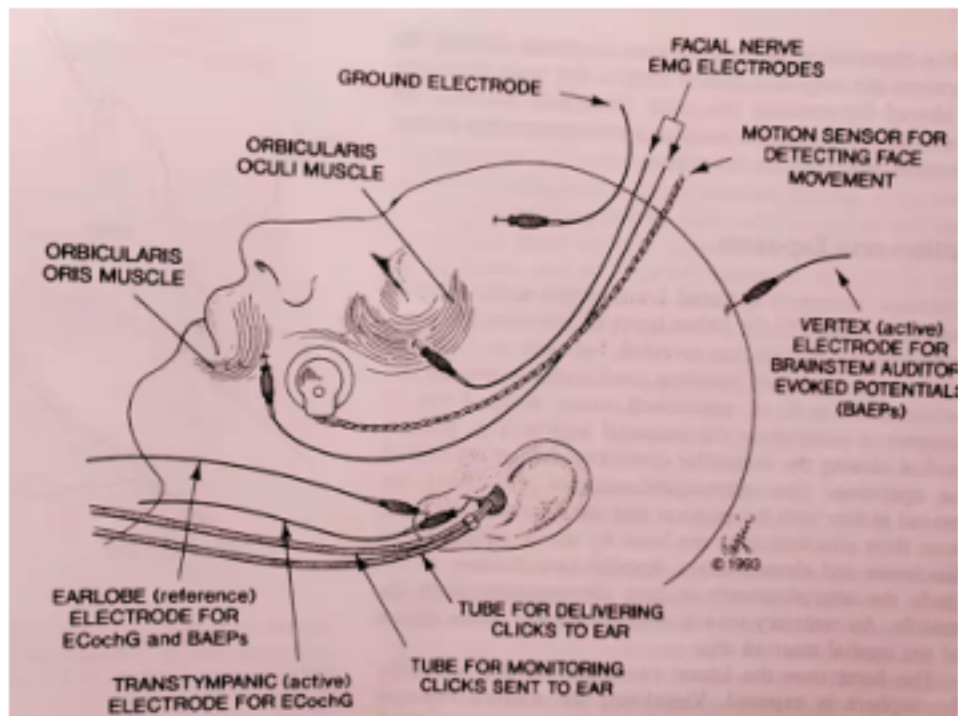


脑干端面神经的寻找





术中电生理监测



关颅的要点

- 内听道口的封闭
- 乳突气房封闭
- 硬膜严密缝合
- 肌肉彻底止血分层缝合

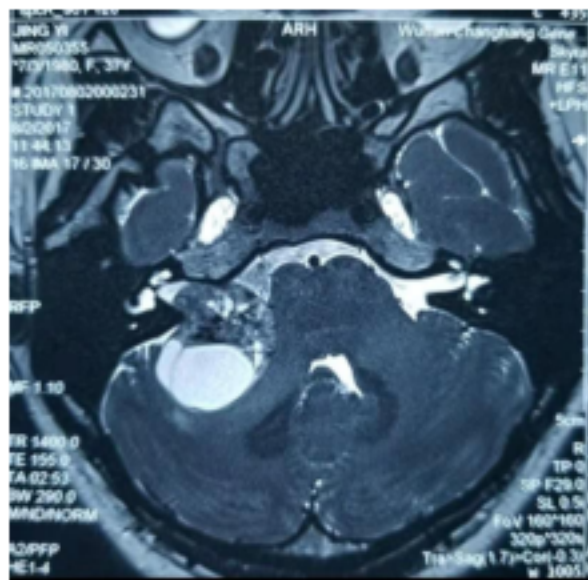
病例分享

病例 1

患者女性，37岁。

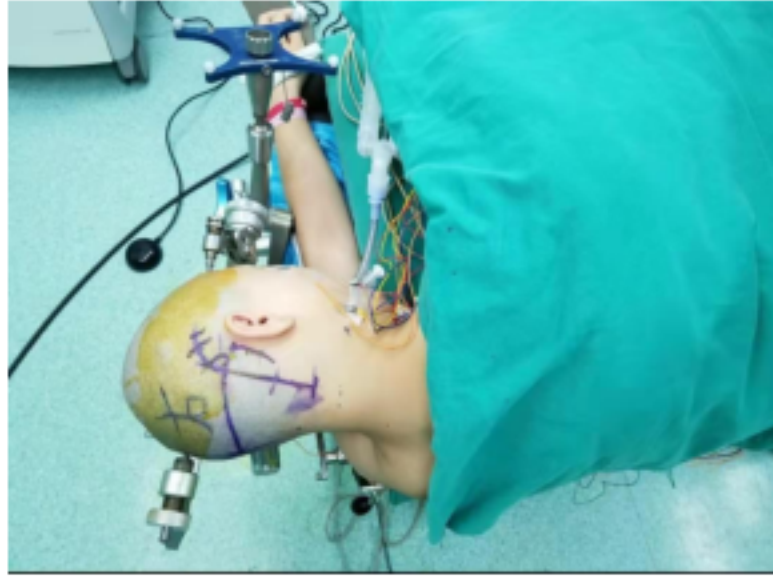
耳鸣一年，加重1月

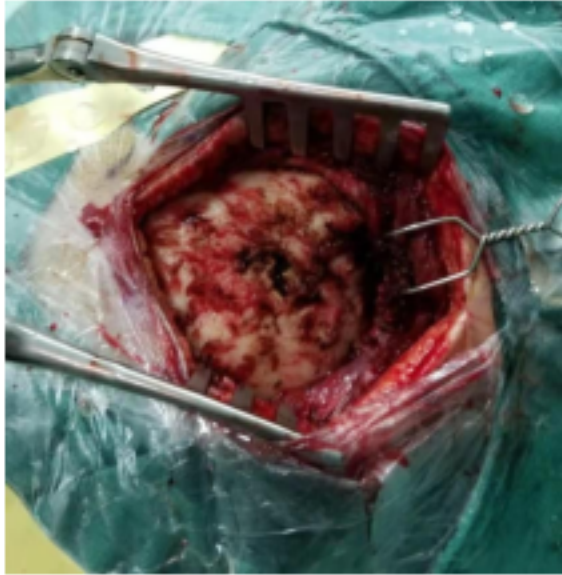
头部MR检查提示：右侧CPA占位，考虑听神经瘤

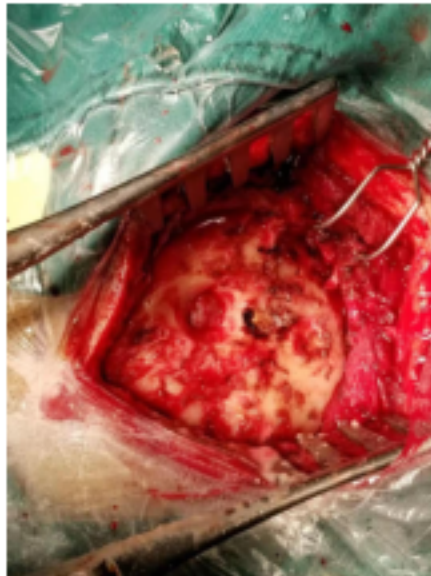


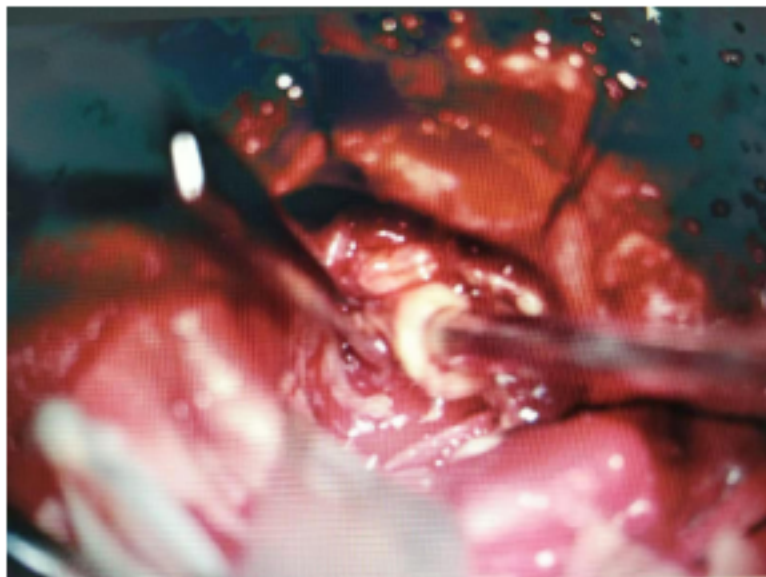




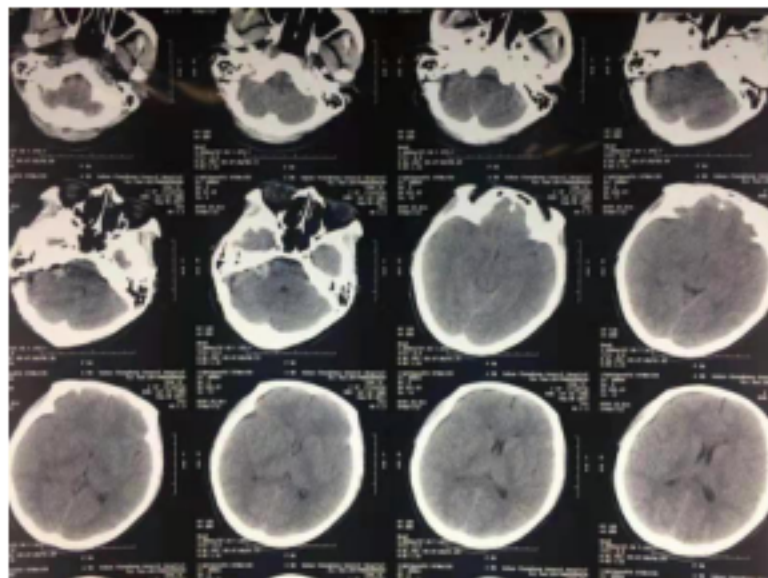


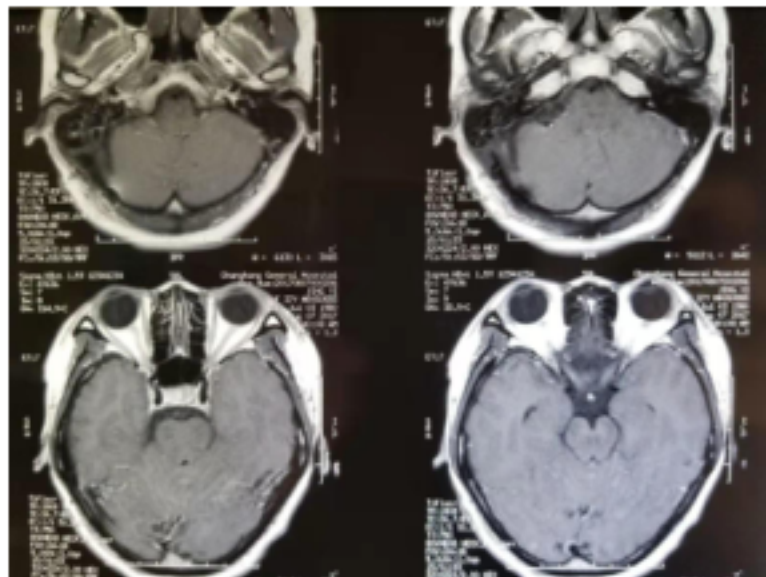










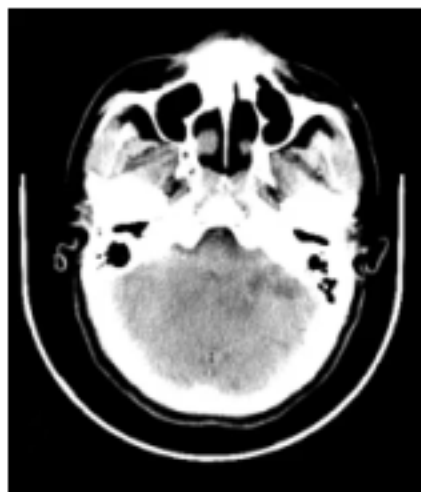


病例 2

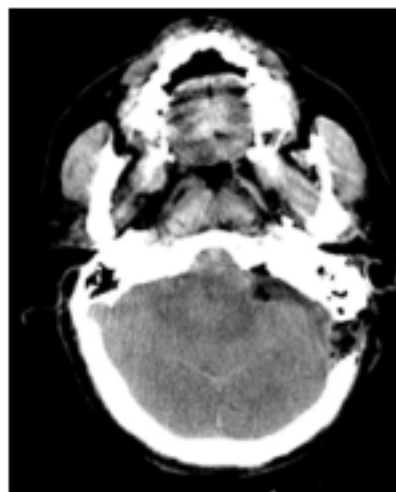
患者男性62岁，

左耳听力下降1月余，外院头部MR提示：左CPA占位

术前头部CT



术后头部CT

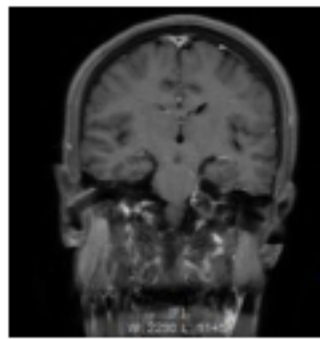
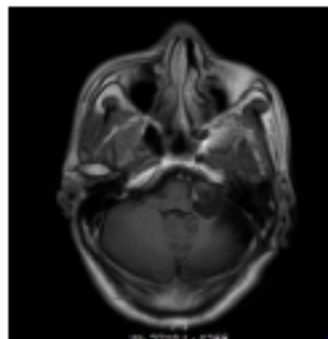


病例 3

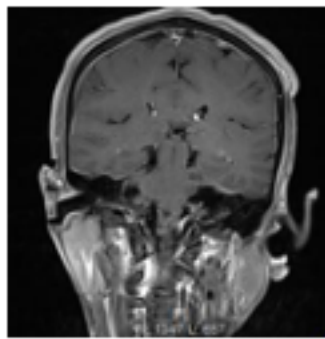
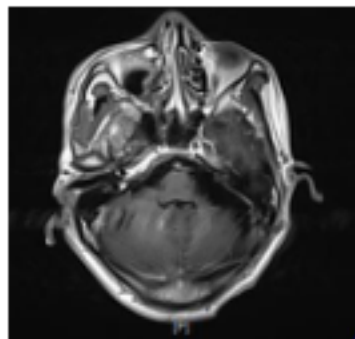
患者男性，49岁

听神经瘤术后15年，饮水呛咳1月余

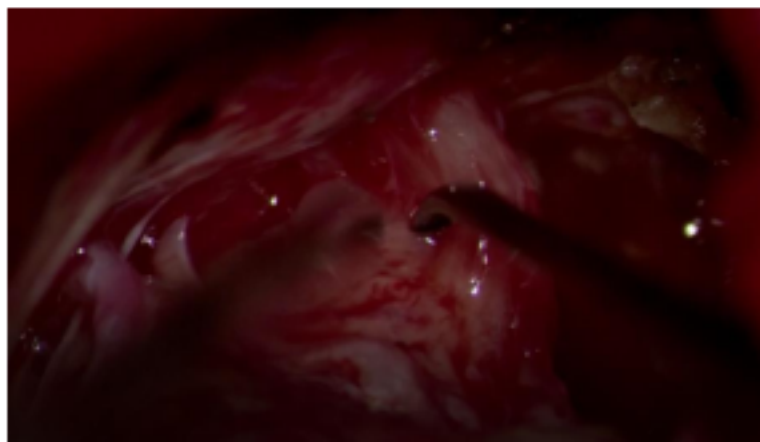
术前MR



术后MR



手术视频

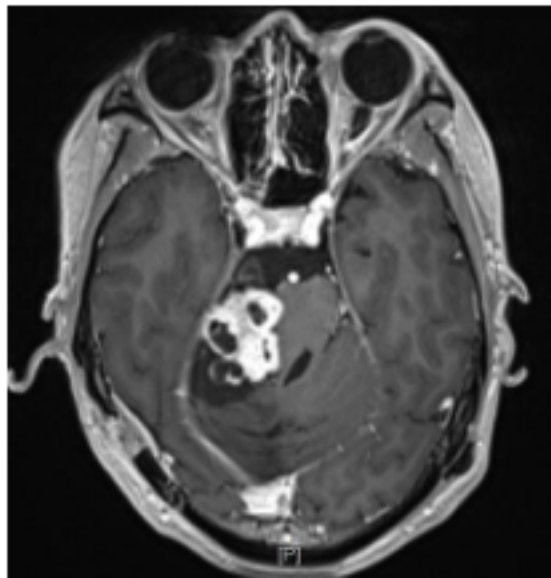
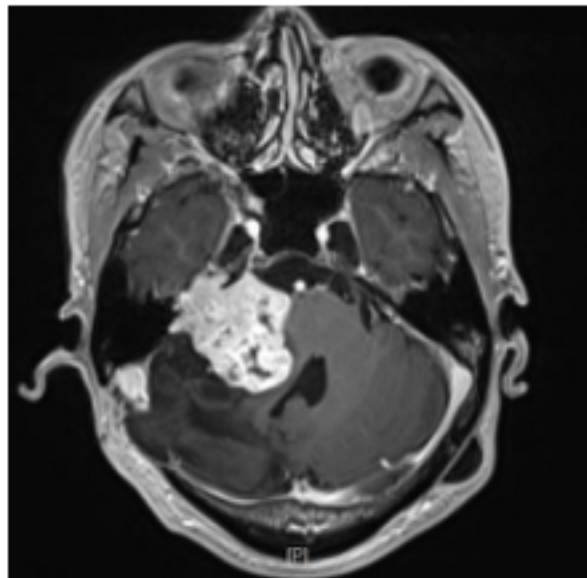


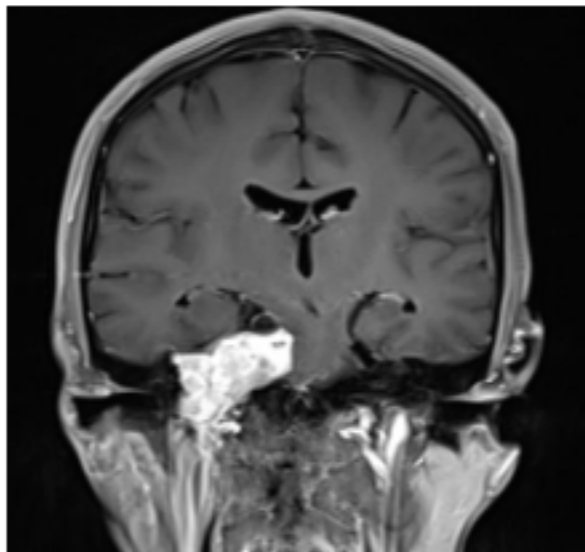
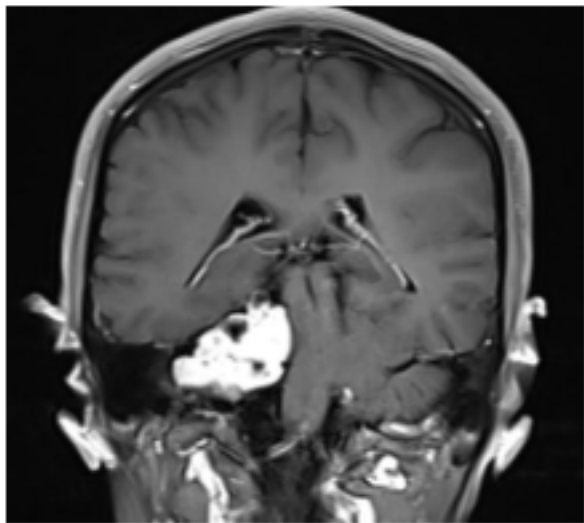
病例4

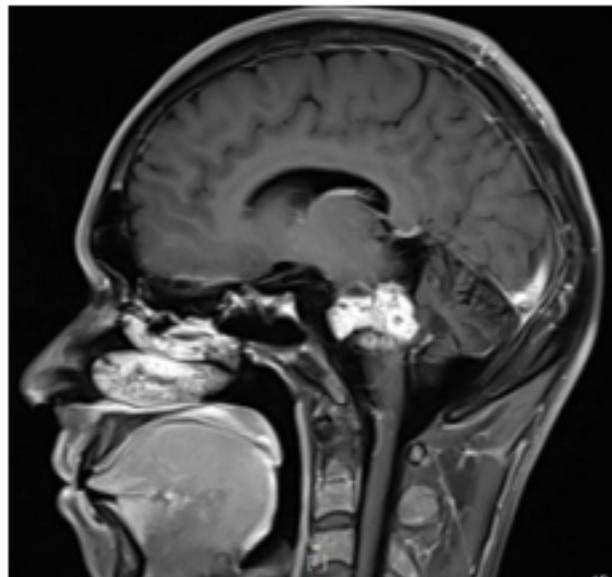
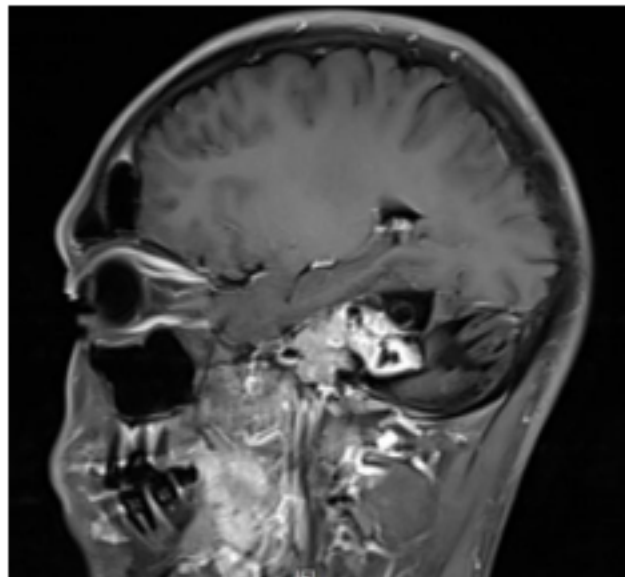
患者，男性40岁

听神经瘤术后11年，间断头痛伴呕吐10月入院

术前MR检查

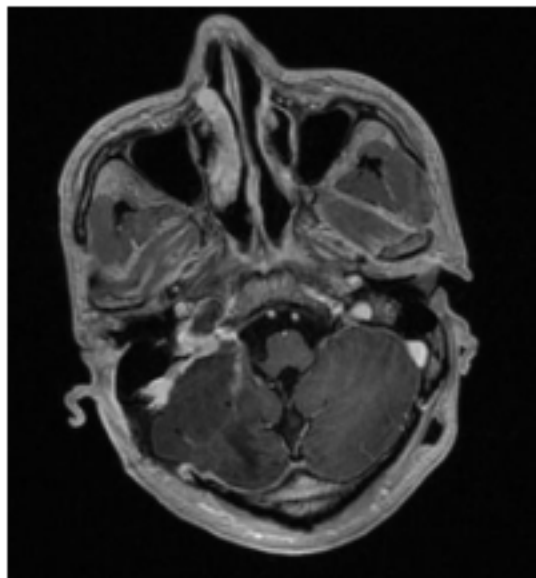
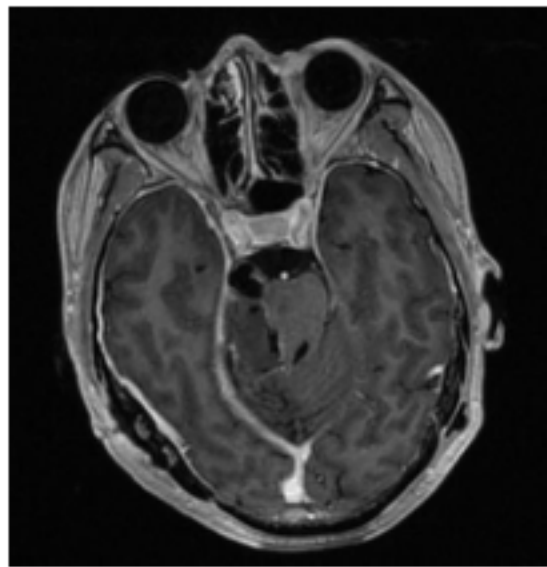


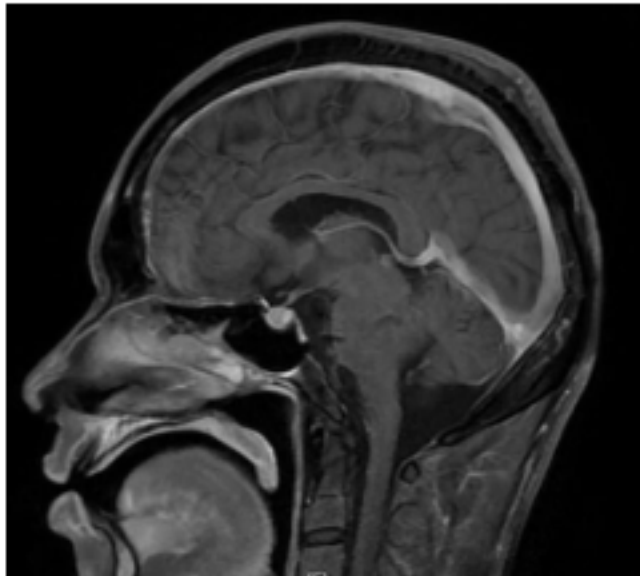
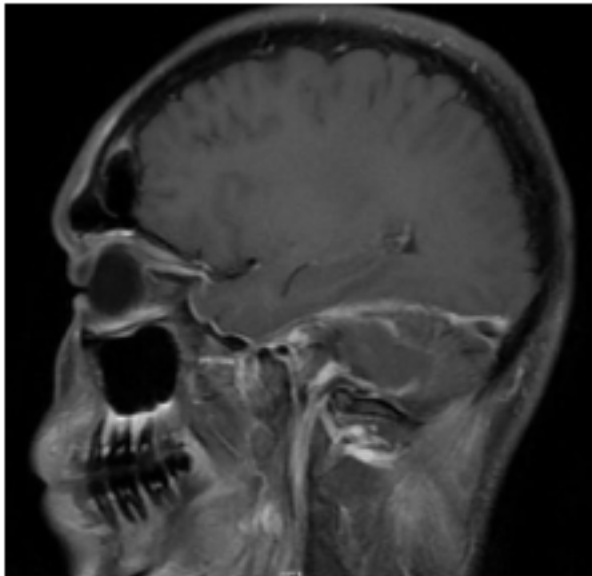






术后MR复查





谢谢聆听!

